جوستاف روسي انترناشيونال - مصر (EPE) "

طلب علاج أول مرة / متردد

إسم المريض رباعياً : صابحه خليفه نصر حسانين
المحافظة : الجيزه **الحي / المدينة :** البدرشين **العنوان :** زاويه دهشور
رقم البطاقة : 27607012105707 **رقم التليفون :** 01148647227 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 01-07-1976 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات الفئ لمرضي الاورام المنتشر [14456] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: مايجه عليه نصر حساني
National ID: 27607012105707

GUSTAVE ROUSSY

تقرير طبي

السيد

تحية طيبة وبعد،،

بناء للطلب المقدم من	بشان تقرير طبي للمريض
بقسم:	رقم طبي:

تتشرف بان توضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :	تاريخ وساعة الخروج :
[illegible] yyyy-mm-dd	PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدي و تحتاج الي Pin Medication And Anti Emetics

التاريخ : 05-07-2026	توقيع : [illegible]
	اختيب المعالج : [illegible]



DMS

ناظلين
ريليود ا



مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

مصر هي صحة

MDT FORM 4 / 3 /2026 G

Name	[illegible]	MRN	2.1C/C.C2
		ID	
Age	55 yrs Old	Phone	C11.V.1C1.0V.V
Menopausal status	Post menopausal	Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity	HTN - DM		

Clinical Data

BX: IDC GIII "Metaplastic"

ER:	PR:	HER 2:	Ki67:
0/8	2/8	-ve	60%

Sonomammography:

Mets Workup:

R L



TNM

Recommendations:

Pt received NAC (DOAC x4 → Taxol / Gabo x12)
ending on 10/12/25 → suggest response favors Inflammatory
P

MDT OUTCOME & PLAN Fe.

① Urgent re-staging (CT CAP & bone Scan).
② Neoadjuvant, adjt to Navelbine / Xeloda ③ re-challenge by Cape / Gabo if progressive response.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
3/3/2026	4/3/2026				

Patient Name	: الأستاذه / صابحه خليفه نصر حسانين	Request No.	: 13008833
Patient No.	: 27607012105707	Request Date	: 15/10/2024
Path No.	: 3933/24	Verification Date	: 30/10/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 48 Y	Department	: Out Patient
Refereing Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.
Nature of Specimen: Tru-cut biopsy, collected at 14/10/2024, 11:00 am.

Gross Examination:
- Three grayish white tissue cores up to 1.2 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for P63, ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
* Histological type: Invasive duct carcinoma (Metaplastic carcinoma).
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+2) Grade III.
* Ductal carcinoma in situ: No.
* Perineural invasion: No.
* Angioinvasion: No.
* Additional pathologic findings: Focal squamoid differentiation.
* IPT (Positive control showed appropriate reactivity):
- P63: Focally positive in tumor cells.
- ER: No nuclear reaction in tumor cells (score 0/8).
- PR: Weak nuclear reaction in 1% of tumor cells (score 2/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 60% nuclear reaction of tumor cells.

Diagnosis:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade III (metaplastic carcinoma) with focal squamoid differentiation.
IPT:

Verified by :
Dr. Sagda El-Kady

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	:الأستاذه / صفيحه خليفه نصر حسانين	Request No.	: 13008833
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Age	:48 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

ERM036720263

- P63: Focally positive in tumor cells.
- ER: Negative (score 0/8).
- PR: Negative (score 2/8).
- Her2: Negative (score 0).
- Ki67: Positive in 60% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, Intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Rana Fathy.
Dr. Sagda Elkady.

Verified by :
Dr. Sagda El-Kady

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qudema - Cairo, Egypt

PATIENT NAME: Sabahah KHALEEFA NASR HASANEN	**PATIENT ID:** 10124253523
GENDER: Female	**AGE:** 48 y
DATE : 19-Nov-2024 05:37 PM	**ORDER:** 3130419,3130579,3130580

Dynamic Contrast Enhanced MRI Study of the Breast

Pulse Sequences:

- Pre-contrast: axial T1,T2,STIR and sagittal T2 WIs.
- DWI and ADC mapping.
- Post contrast: axial VIBE and Subtraction WIs

****complementary mammography and US were done.**

Indication:

- Pathological right axillary lymph nodes (pathologically proven metastatic of breast origin) to detect any occult breast lesions.

Report:

- Prominent fibroglandular parenchyma of both breasts showing mild nodular back ground parenchymal enhancement.

Right breast:

- Right breast outer quadrants (upper and lower) and deep retroareolar asymmetrical mild heterogenous parenchymal enhancement, more appreciated in delayed phases, corresponding to focal asymmetry on mammography. Complementary US revealed subtle parenchymal alteration with no definite masses.
- Right LOQ small circumscribed oval nodule with intrinsic fat signal measuring about 7x6mm?? IMLN.
- Multiple pathological partially amalgamated right level I,II and III axillary lymph nodes, the largest at level I measuring about 44x31mm. Multiple pathological supra clavicualr lymph nodes (seen by US).

Left breast:

- No enhancing lesions are seen, therefore, no time signal intensity curve was measured.
- Left UOQ tiny rounded rim enhancing nodule measuring about 6mm.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymphadenopathy.

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well as its main tributaries.

- Gall bladder stone is noted with no peri-cholecystic fluid.
- Left adrenal gland small fat-containing lesion measuring 7mm, likely myelolipoma.
- Normal appearance of the pancreas, spleen (and small adjacent splenules are noted), kidneys (apart from left renal small upper pole cyst), right adrenal, aorta, I.V.C and the pelvic organs.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic masses or collections.
- **The revised bone window revealed:**

1. Multilevel vertebral bodies tiny lucent foci.
2. Dorso-lumbar spondylo-degenerative changes.
3. No CT evidence of vertebral collapse or extra-osseous soft tissue component.

Opinion:

- **Right breast UOQ few enhancing nodules with pathologically enlarged right axillary as well as supraclavicular lymph nodes. For correlation with sonomammography findings.**
- **No definite pulmonary or hepatic metastatic deposits.**
- **Multilevel vertebral bodies tiny lytic foci for bone scan assessment.**

Reported By,
Toka Elbehairy,MSc
Dr. Mahmoud Ibrahiem, MD **18-Nov-2024 01:12 PM**

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Women's Health Outreach Program

Name: sabha khalifa nasr
Date: 14-10-2024

Bilateral Full field Digital Mammography

Clinical summary: Long standing history of right breast lump, which is now painful

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Heterogeneously dense breast parenchyma (ACR density c).
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Multiple pathologically enlarged axillary lymph nodes
- **Ultrasound** demonstrated No solid or cystic lesions.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- **Multiple enlarged axillary angulated lymph nodes, all showing eccentric fatty hilum , the largest seen measuring about 4x3 cm.**

Left breast

- Heterogeneously dense breast parenchyma (ACR density c).
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/subcutaneous tissue and nipple/areolar complex
- Non-specific axillary LNs.

Impression:

Right breast Normal breast study, with suspicious axillary lymph nodes .
(BIRADS 4) for histopathology correlation (biopsy was taken)
Left breast (BIRADS 1): Normal study.

Best regards,
Dr. Norhan Osama , MD

4

Opinion:

- A case of pathological axillary lymph nodes (pathologically proven metastatic of breast origin) showing right outer quadrants and retroareolar low suspicious parenchymal enhancement for biopsy (BIRADS 4).
- Right LOQ probably benign looking nodule (BIRADS 3).
- Left UOQ probably benign nodule (BIRADS 3).

Reported By,
Dr, Yara Alaa, MSc
Prof.Dr. RASHA KAMAL 20-Nov-2024 04:02 PM

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PATIENT NAME: Sabahah KHALEEFA NASR HASANEN
GENDER: Female
DATE : 17-Nov-2024 02:30 PM

PATIENT ID: 10124253523
AGE: 48 y
ORDER: 3125992,3125993

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion.

FINDINGS:

1-CHEST

- Right breast UOQ multiple small enhancing nodules; the largest measures 7 mm in diameter.
- Multiple pathologically enlarged right axillary level I, II and fewer level III lymph nodes; the largest is at level I measuring 24 x 34 mm.
- Pathologically enlarged right supraclavicular lymph nodes; the largest measures 15 x 13 mm.
- Tiny right internal mammary lymph node is noted measuring 5 mm.
- Few subcentimetric left axillary level I and II and supraclavicular lymph nodes are noted.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathologically enlarged mediastinal or hilar lymph node.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- Mildly enlarged liver with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and is main branches as

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مركز بهية للجود
شارع عليوة أول شارع الهرم بجوار مترو الجيزة
ميدان الجيزة - أمام أول شارع الهرم بجوار مترو الجيزة

Patient's Name:	**Sabha Khalifa Nasr Hassanain**
Date:	**2026-08-24**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

CLINICAL INDICATION: breast cancer swollen arm.
COMPARISON: No previous study for comparison.
TECHNIQUE: Multiplanar CT with IV contrast neck chest abdomen pelvis.

FINDINGS:
NECK:
The nasopharynx, oropharynx and hypopharynx are preserved, with no obvious mass lesion.
The endolaryngeal structures that include the epiglottis, aryepiglottic folds, false and true vocal cords appear intact.
The laryngeal skeleton that includes the thyroid and cricoid cartilages as well as the hyoid bone appear intact.
The examined salivary glands that include the parotid, sub-mandibular and sub-lingual salivary glands appear intact.
The thyroid gland appears of normal size with no obvious thyroid nodule.
The carotid arteries and internal jugular veins appear intact.
The cervical musculature and visualized osseous structures appear intact.
Right subcentimetric multiple cervical lymphadenopathy.

CHEST

FINDINGS:
right breast marked skin and subcutaneous thickening and edema involving right breast, visualized part of right neck right chest and abdominal wall.
Right breast is edematous with multiple focal lesions
Multiple right axillary lymph nodes enlargement reaching 3 cm in short axis.
right pectoralis muscle major and minor are seen enlarged edematous, loss of fat planes with heterogenous enhancement of ill-defined area 4.5 x4 cm.
no evidence of rib involvement or destruction.
TRACHEA: Normal diameter and wall thickness, with no evidence of stenosis.
BRONCHI: Patent, with no evidence of bronchiectasis.
LUNGS: No pulmonary nodules, alveolar infiltration or interstitial thickening.
Right upper lobe posterior pleural thickening and nodularity with ground glass opacity
MEDIASTINUM: No mediastinal mass lesion or lymph node enlargement.
HILUM: right subcentimetric lymph node enlargement.
CARDIAC CHAMBERS:
No chamber dilatation.
PERICARDIUM: No pericardial thickening or effusion.
AORTA (ascending, arch and descending): Normal diameter, with no evidence of aortic aneurysm.
PULMONARY TRUNK AND MAIN PULMONARY ARTERIES: Normal diameter, with no evidence of pulmonary hypertension.
PLEURA: No pleural effusion
Right upper lobe posterior pleural thickening and nodularity with ground glass opacity
CHEST WALL: spondylo-degenerative changes.

Patient's Name:	**Sabha Khalifa Nasr Hassanain**
Date:	**2026-08-24**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

LIVER:
Size: average.
Surface: Smooth and regular.
Hepatic focal lesion: No solid or cystic mass lesion.
Portal vein: Patent.
Hepatic biliary system: Not dilated.
GB single large stone
SPLEEN: moderate enlarged 16 cm. No splenic focal lesion.
KIDNEYS: Normal size, parenchymal thickness, density and enhancement. Normal contrast excretion on delayed images. No calculi or back pressure changes on either side. The perinephric fat planes are preserved.
ADRENAL GLANDS: left adrenal shows small 2 hypodense lesions with HU less than 10 HU, likely adenomas
Pancreas: Normal size and density. No pancreatic mass lesion. The pancreatic duct is not dilated.
ABDOMINAL AORTA: Unremarkable. No aneurysmal outpouching or dissection.
IVC: Unremarkable.
LYMPH NODES: No retroperitoneal, mesenteric or pelvic lymphadenopathy.
SMALL AND LARGE BOWEL: deudenal wall thickening is noted
large bowel long segment 8 cm in length proximal to rectum showing luminal narrowing with thickened wall 6 mm.
URINARY BLADDER: increase wall thickness denoting cystitis. No intravesical stone or mass.
RECTUM: Normal wall thickness. The perirectal fascia is preserved.
REPRODUCTIVE SYSTEM: bilateral prominent adnexa right reaching 4 cm.
PERITONEUM: No ascites.
Umbilical hernia passing bowel loop.
BONE: spinal spondylo-degenerative changes.

- **A case of right breast cancer with pathological right axillary lymph nodes and invasion of right pectoralis major and minor with no evidence of bone invasion. Right-sided neck, breast, chest and abdominal wall edema. Evaluation of for venous thrombosis is limited on this routine examination. Dedicated venous Doppler ultrasound and/or CT venography is considered.**
- **Right upper posterior pleural thickening and nodularity. PET CT is recommended.**
- **Bilateral enlarged ovaries. correlation with transvaginal ultrasound is recommended.**
- **Segmental bowel wall thickening. clinical correlation and further assessment are recommended.**
- **GB stone – cystitis -non-complicated umbilical hernia**

Dr.Nada Nasser,FEBR

	Sabha Khalifa Nasr		15-8-2026
	Dear Colleague		

MRI OF THE CERVICAL SPINE

TECHNIQUE:

- Sagittal T1 and T2 WIs
- Axial GE WIs

FINDINGS:



- Straightened cervical curve.
- Normal dimensions of cervical spinal canal.
- Cervical spondylosis evident by small marginal osteophytosis of the vertebral endplates and degenerated discs.
- C3-4 posterior disc protrusion is seen encroaching upon the CSF space and indenting the cord.
- C4-5 mild leftposterolateral disc bulge is seen encroaching upon the CSF space and not indenting the cord.
- C5-6 posterior and right posterolateral disc bulge is seen encroaching upon the CSF space and indenting the cord yet partially encroaches upon the right neural exit foramen.
- C6-7 posterior disc bulge is seen encroaching upon the CSF space and indenting the cord yet partially encroaches upon the right neural exit foramen.
- C5-6 and C6-7 bilateral neurocentral arthropathy.
- Normal size and signal of the cervical cord.
- No marrow infiltrations.
- Intact cranio-cervical articulation.
- No para-spinal soft tissue masses or collection.

OPINION:

- **Cervical spondylosis with multi-level disc lesions; as described.**

Dr.Ahmed Shawky,MD

Name : Sabha Khalifa Nasr Hassanain
Branch : Dokki
Date : 20-09-2026 08:49
Medical No. : 461471

MRI OF THE RIGHT SHOULDER AND PROXIMAL ARM



Technique:

Multiplanar multisequence MRI of the right shoulder and proximal arm including T1-weighted and fluid-sensitive/STIR sequences, with post-contrast images as available.

Findings:

- Diffuse extensive subcutaneous soft-tissue edema involving the visualized right shoulder and arm, with reticular/infiltrative appearance.
- The edema is predominantly within the subcutaneous tissues, with no definite well-defined or discrete solid soft-tissue mass identified in the visualized arm images.
- Mild associated intermuscular fascial edema is noted. No convincing focal intramuscular mass lesion is demonstrated on the available sequences.
- Multiple pathologically enlarged right axillary lymph nodes, likely nodal metastatic disease.
- No definite focal marrow-replacing lesion or cortical destruction of the visualized humerus.
- No significant focal abnormality of the visualized muscles apart from the associated edema.
- No organized fluid collection or abscess is identified.
- The visualized shoulder joint shows no gross joint effusion or destructive osseous lesion.

Impression:

Known right breast carcinoma showing multiple right metastatic axillary lymph nodes and diffuse extensive subcutaneous edema of the visualized right shoulder/proximal arm, with associated intermuscular fascial edema, most consistent with lymphedematous/infiltrative soft-tissue edema, particularly in the setting of right breast carcinoma and axillary nodal disease.

Much Obliged;
Dr.Marwa Elgawish, MSc, FEBR

مستشفى روشي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

09/23/2026

MDT FORM

Name:	Sabeha / Nasr Hassanien	**MRN:**	7344
Age:	50 Y	**ID:**	27607012105707
Menopausal Sta	postmenopausal	**Phone:***	01148647223
NRS SCORE:		**Marital Status:**	married
BMI:		**Family History:**	NO
Gender			
CO Morbidity:	DM , HTN	**Reason Of MDT:**	progressive
Surgical History:*	No	**ECOG Performance Status**	○ 0 ● 1 ○ 2 ○ 3 ○ 4

Clinical Data:

BX: IDC GIII

ER: 0/8 **PR:** 2/8 **HER2:** -ve **KI67:** 60

History:

55 Y/O, postmenopausal, DM/HTN.
Rt breast IDC G3, ER 0/8, PR 2/8, HER2?, KI-67 60%, initially cN2.
Received NACT: 4 AC ? 12 weekly Taxol/Carbo, with poor/sluggish response. Subsequently shifted to Navelbine + Xeloda, with further clinical/radiological progression.
Current: Marked symptomatic locoregional progression with severe pain, pathological Rt axillary LNs and invasion of pectoralis major/minor ? unresectable disease.
for MDT :
-next line of ttt
-for palliative RT to the breast/chest wall ± axilla for pain and local control.
???
- bone scan >> R
-germline BRCA1/2 >> R

R L

TNM

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

Immunohistochemistry:

Biomarker/Molecular:

Meta Workup:

Recommendations:

MDT Outcome & Plan

For:
1.PDL1 & BRCA testing
2.In view of high burden advanced locoregional disease, shift to Gemzar/Cisplatin.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

23/9/2026

23/9/2026

Patient's Name:	**Sabha Khalifa Nasr Hassanain**
Date:	**2026-08-24**
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	Sabha Khalifa Nasr		15-8-2026
	Dear Colleague		

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- Cervical spondylosis evident by small marginal osteophytosis of the vertebral endplates and degenerated discs.
- C3-4 posterior disc protrusion is seen encroaching upon the CSF space and indenting the cord.
- C4-5 mild leftposterolateral disc bulge is seen encroaching upon the CSF space and not indenting the cord.
- C5-6 posterior and right posterolateral disc bulge is seen encroaching upon the CSF space and indenting the cord yet partially encroaches upon the right neural exit foramen.
- C6-7 posterior disc bulge is seen encroaching upon the CSF space and indenting the cord yet partially encroaches upon the right neural exit foramen.
- C5-6 and C6-7 bilateral neurocentral arthropathy.
- Normal size and signal of the cervical cord.
- No marrow infiltrations.
- Intact cranio-cervical articulation.
- No para-spinal soft tissue masses or collection.

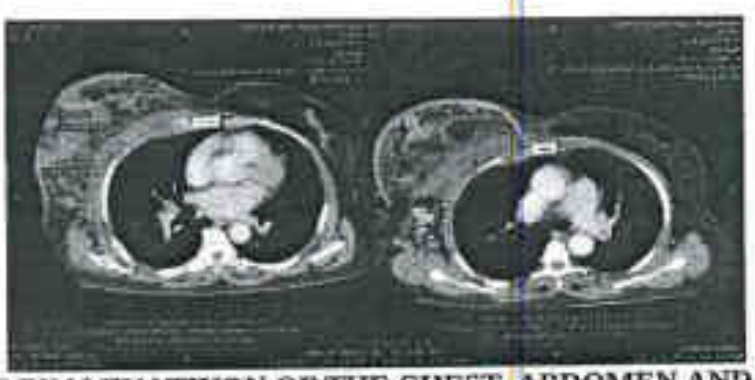
OPINION:

- **Cervical spondylosis with multi-level disc lesions; as described.**

Dr.Ahmed Shawky,MD

Patient's Name: **Sabha Khalifa Nasr Hassanain**

Date: **2026-07-13**
Referred by: Prof.Dr. **no doctor**
Hospital: **Goustave Roussey International Egypt**



MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-
as compared to previous study dated 26/4/2026, the current study showed

- Relatively regressive course of the right breast retroareolar soft tissue hetrogenous mass lesion measures 7.6x5.3cm compared to 10.4x4cm at the previous study
- still seen two enhancing lobulated soft tissue mass lesions are seen in the external aspect of right breast(UOQ and LOQ respectively) and each measures about 21x19mm and 32x19mm in dimensions respectively. The overlying skin is diffusely thickened and reaches to about 12mm internally.
- Multiple variable sized enhancing right axillary and retro-pectineal lymph nodes are seen with loss of their hilar fat contents and the largest one measures about 24x24 mm in dimensions compared to 34x25mm at the previous study.
- Normal left breast.
- Multiple variable sized benign-looking left axillary lymph nodes are seen with preservation of their hilar fat contents and the largest one measures about 18x6.3mm in dimensions.
- Hyperinflated both lungs.
- No sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidations or calcifications noted.
- Enhancing pretracheal and measure about 27x18mm in dimensions.

Patient's Name: **Sabha Khalifa Nasr Hassanain**

Date: **2026-07-13**
Referred by: Prof.Dr. **no doctor**
Hospital: **Goustave Roussey International Egypt**

- No other sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- **Normal bones of the thoracic cage apart from mild spondylo-degenerative changes of the scanned spine and diffuse osteoporosis.**
- Border line sized liver,measuring about 15.6 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- A stone of mixed density is seen in the gall bladder fossa,measuring about 14x13mm with its CT density of 109HUs.
- Average sized spleen,measuring about 8.1 cm. No focal lesions seen inside.
- Relatively thickened left adrenal gland.
- Normal CT appearance of the pancreas, functioning both kidneys, right adrenal gland,aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- Bilateral ovarian cyst is seen ,larger on the right side and measures about 28x26mm in dimensions.
- The rest of the pelvic structures are unremarkable.

IMPRESSIONS:

- **Relatively regressive course of the right breast masses & right axillary LNs for sonomammography**

- **Stationary rest of study**

Prof/Hanan El-Ahwal.

Patient's Name:	**Sabha Khalifa Nasr Hassanain**
Date:	**2026-08-09**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: right breast carcinoma underwent NACT , follow ups study.

Findings:

Right breast.

- The entire breast shows thickened skin associated with increases parenchymal density compared to the left side
- large infiltrating mass involving most of the breast
-
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Multiple pathological axillary level I and II with globular shape and lost hilum , the largest measures 15x 12 mm
- multiple pathological supraclavicular lymph nodes , the largest measures 13 x10 mm

Left breast.

- Heterogenously dense breast parenchyma that may obscure small lesions. (ACR c)
- no assymetry, masses, speculations, or pathological microcalcifications
- **Ultrasound** demonstrated no solid or cystic masses .
- Normal skin/nipple/areola complex.
- Non specific axillary nodes

Conclusion:-

- ***Right breast (BIRADS 6): comparing to the radiological available reports, progressive disease***
- ***Left breast (BIRADS 1): normal study***

dr farah elshobaky

Patient's Name: **Sabha Khalifa Nasr Hassanain**

Date: **2026-07-13**
Referred by: Prof.Dr. **no doctor**
Hospital: **Goustave Roussey International Egypt**



MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-
as compared to previous study dated 26/4/2026, the current study showed

- Relatively regressive course of the right breast retroareolar soft tissue hetrogenous mass lesion measures 7.6x5.3cm compared to 10.4x4cm at the previous study

- still seen two enhancing lobulated soft tissue mass lesions are seen in the external aspect of right breast(UOQ and LOQ respectively) and each measures about 21x19mm and 32x19mm in dimensions respectively. The overlying skin is diffusely thickened and reaches to about 12mm internally.
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- Normal left breast.
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- Hyperinflated both lungs.
- No sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidations or calcifications noted.
- Enhancing pretracheal and measure about 27x18mm in dimensions.

Patient's Name: **Sabha Khalifa Nasr Hassanain**

Date: **2026-07-13**
Referred by: Prof.Dr. **no doctor**
Hospital: **Goustave Roussey International Egypt**

- No other sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- **Normal bones of the thoracic cage apart from mild spondylo-degenerative changes of the scanned spine and diffuse osteoporosis.**
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- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
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- Normal CT appearance of the pancreas, functioning both kidneys, right adrenal gland,aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- Bilateral ovarian cyst is seen ,larger on the right side and measures about 28x26mm in dimensions.
- The rest of the pelvic structures are unremarkable.

IMPRESSIONS:

- **Relatively regressive course of the right breast masses & right axillary LNs for sonomammography**
- **Stationary rest of study**

Prof/Hanan El-Ahwal.

GUSTAVE ROUSSY
INTERNATIONAL

NAME: sabha khalifa naser
DATE:7/6 /2026

Duplex Examination of the venous system of right upper limb revealed: -

- *The subclavian, axillary, radial & ulnar veins of the examined upper limb all are patent, totally compressible with normal color filling & preserved respiratory variation.*
- ***No evidence of D.V.T.***
- *Patent cephalic &basilic veins along their course* ***,no evidence of superficial thrombophlebitis at time of scan***

OPINION: -

No evidence of DVT or superficial thrombophlebitis at time of scan
Mild interstitial edema of the examined limb.

Best regards
Dr. NERMEEN RAMADAN FEBR. MD

٧٧٤٤ معطيه بالسبعة

جوستاف روسي

Patient's Name:	**SABHA KHALIFA NASR HASSANIN**
Date:	**2026-04-26**
Referred by:	Prof.Dr. **CT CAP IV**
Hospital:	**Goustave Roussey International Egypt**

MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH CONTRAST

TECHNIQUE: Multi slice axial CT images of the chest, abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images. I.V contrast administration.

FINDINGS:

CECT CHEST:

****Compared to previous study dated on 4-2-2026:**

- **Progressive course** as regard,
  - size of the previously seen right breast retro areolar soft tissue heterogenous mass lesion measures 10.4 x 4 cm compared to 5.4 x 4 cm in the previous study.
  - Size of the largest right axillary lymph node measures 3.8 x 3 cm compared to 3.4 x 2.5 cm in the previous study.
- Still, bilateral basal atelectasis.
- No cavitary lesions or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Bilateral elastofibroma dorsi.

CECT ABDOMEN AND PELVIS:

- The liver is average in size showing homogenous CT density with no enhancing focal lesions or biliary radicles dilatation.
- The gall bladder shows average wall thickness. Its cavity shows a dense stone measures 1.4 cm.
- Normal CT appearance of the spleen, pancreas and both kidneys.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No abdominal or pelvic pathologically enlarged lymph nodes.
- No free collection.

THE SCANNED SKELETON:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

Patient's Name:	**SABHA KHALIFA NASR HASSANIN**
Date:	**2026-04-26**
Referred by:	Prof.Dr. **CT CAP IV**
Hospital:	**Goustave Roussey International Egypt**

OPINION:

****Compared to previous study dated on 4-2-2026:**

- **Progressive course** as regard,
  - **size of the previously seen right breast retroareolar soft tissue heterogenous mass lesion.**
  - **Size of the largest right axillary lymph node.**
- **Still, bilateral basal atelectasis.**
- **No hepatic or osseous lesions.**

Dr.Heba El Balshy.MD
Lecturer and Consultant of Diagnostic and Interventional Radiology.
SLI-Menoufia University

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

٧٣٤٤

Name : Sabha Khalifa Nasr Hassanain	Date : 06-04-2026 09:59
Branch : Dokki	Medical No. : 343637
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : تعاقد مستشفيات

ISOTOPIC BONE SCAN REPORT

Indication: 55-year-old female with history of breast carcinoma, presenting with bone aches. Evaluation for possible osseous metastatic disease.

Imaging Protocol: Whole-body planar images (anterior and posterior projections) acquired approximately 2–3 hours post intravenous injection of the tracer (Tc-99m MDP).

FINDINGS:

- No abnormal radiotracer uptake suggestive of skeletal metastatic disease could be identified.
- Degenerative/arthritic changes are noted in the spine, sternum, both shoulders, and weight-bearing joints.

IMPRESSION:

- No scintigraphic evidence of osseous metastases.
- Follow up as per clinical pathway.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

NILE DIAGNOSTIC CENTER

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الست المصرية هي صحة مصر

MDT FORM 5/11/2025 G

Name	طايب نعيمة نظير	MRN	E·19/C122
		ID	10124253523
Age	55 yrs old	Phone	01140433b8
Menopausal status	Post menopausal	Marital status	Married
NRS SCORE		Family history	-ve
BMI		REASON OF MDT	cT2.N1c1.0.N
CO morbidity	HTN & DM		

Clinical Data

BX: IDC GIII

ER:	PR:	HER 2:	Ki67:
0/8	2/8	-ve	60%

Sonomammography:
Rt Br normal study 10/2024
+ suspicious AUNs



Mets Workup: MRI Rt Br → Rt Br metastasis & Parenchymal suspicious lesion
- CT CAP → multiple variable sized lytic lesions
→ Path. enlarged supraclavicular LNs 15x13 mm
11/2024 TNM 03c
for bone scan

Recommendations: Bone scan → free
Pt received NACT DD AC x4 & Taxol/Carbo x12 weekly
for surgical Review regarding Rt MRM & plan of management
as per last (MDT Surgical Review)
Discharge 26/10/2025
* For new Sonomammo to assess Response
4 supra clavi LN
Still skin edema + Axillary LN.
then decision of Rt MRM

MDT OUTCOME & PLAN

Last cycle: 26/10/2025 Omnitarg 26/10/2025

For new Sonomammography.

NEW CAPS SIGNATURE if Not metastatic for Rt MRM

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
		Mona Tawfik			

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الست المصرية هي صحة مصر

VYEE

MDT FORM 5/11/2025 G

Name	مريت خليفه نادر	MRN	E.19/C155
		ID	10124253523
Age	55 yrs old	Phone	01140433b8
Menopausal status	Post Menopausal	Marital status	Married
NRS SCORE		Family history	-ve
BMI		REASON OF MDT	CA?. V. ICI'ON.)
CO morbidity	HTN & DM		

Clinical Data

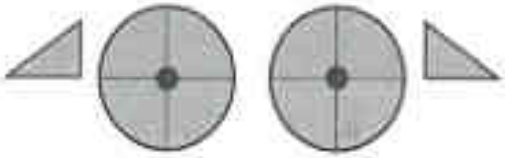
BX: IDC GIII.

ER:	PR:	HER 2:	Ki67:
0/8	2/8	-ve	60%.

Sonomammography:
Rt Br normal study 10/2024
+ suspicious AUNs

Mets Workup: MRI Rt Br → Rt Br Retro orbital Parenchy (suspicious) & 11/2024
- CT CAP → multiple U. bytic lesions TNM 03c
→ Path. enlarged supraclavicular LNs 15x13 — for bone scan

R L



Recommendations: Bone scan → free
Pt received NACT DD AC x4 & Taxol/Carbo x12 weeks
for surgical Review regarding Rt MRM & plan of management
as per last MDT (Surgical Review)
Discharge 26/10/2025
* For new Sonomammo to assess Response
& supra clavi LN
Still skin edema + Axillary LN
then decision of Rt MRM
Last Cycle: 26/10/2025. Ommaya 24/12/25

MDT OUTCOME & PLAN

For new Sonomammography.

New CAPS SIGNATURE if Not metastatic for Rt MRM

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
		Mona			

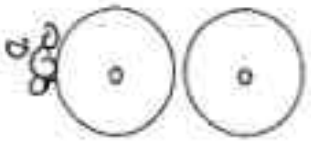
Preventive Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

2-Patient ID:
1- Name : صابحه طليقه نصر حسانين
Sex: Female
3- Medical number: 83243886
Old Med : 4012-2024

I-ID number:
Patient Age : 48 Y



Family history		Phone	01140433468	BMI		REASON OF MDT	
marital status		NRS Score		GROUP		Date of MDT	2025-02-27
Co morbidity	No	Menopausal status		Initial STAGE			

Clinical data

Breast Imaging :	Mets Workup :
rt breast normal study r suspicious LN	CTCAP FREE nri breast: pathological LN rt breast outer & Retro areolar quadrants suspicious parenchyma, LEVEL I LEVEL II, III AXILLA , SUPRACLAV BONE SCAN FREE

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC gIII	0/8	2/8			60

T	N	M
CTX ALTERED PARENCHYMA,	3C	0

Sonomammography

PATIENT HISTORY
ECHO, LABS

MDT OUTCOME & PLAN

NACT DDACX4 WITH GCSF SUPPORT THEN TAXOL CARBOX12 FOLLOWED BY RT MRM

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

Patient Name	:الأستاذه / صفيحه خليفه نصر حسانين	Request No.	: 13008833
Patient No.	:27607012105707	Request Date	: 15/10/2024
Path No.	:3933/24	Verification Date	:30/10/2024
Sex	:Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	:48 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

ERM036728263

- P63: Focally positive in tumor cells.
- ER: Negative (score 0/8).
- PR: Negative (score 2/8).
- Her2: Negative (score 0).
- Ki67: Positive in 60% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Rana Fathy.
Dr. Sagda Elkady.

Verified by :
Dr. Sagda El-Kady

غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

السد المصرية هى صحة مصر

MDT FORM / / 20 G

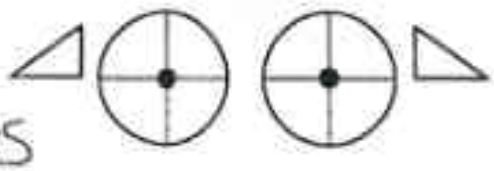
Name	[illegible]	MRN	
		ID	
Age	55 ys old.	Phone	
Menopausal status	post menopausal	Marital status	Married → 3 offsprings.
NRS SCORE		Family history	-ve FH.
BMI		REASON OF MDT	
CO morbidity	HTN → Surgically free.		

Clinical Data

BX: IPC GIII

ER:	PR:	HER 2:	KI67:
0/8	2/8	-ve	60%.

R L



Sonomammography:
Rt. Br. - normal study + Suspicious ALNs.

Mets Workup:
CT CAP+Contrast)
↳ multiple vertebral lytic lesions for Bone scan

Rt - GNM 03c X

Recommendations: enlarged Rt. Supra-clav. For C/S.
Bone Scan → requested.
MRI Breast ← pathological ALNs.
Rt Br. - outer & retroareolar quadrants Suspicious parenchyma.

MDT OUTCOME & PLAN

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

well as its main tributaries.

- Gall bladder stone is noted with no peri-cholecystic fluid.
- Left adrenal gland small fat-containing lesion measuring 7mm, likely myelolipoma.
- Normal appearance of the pancreas, spleen (and small adjacent splenules are noted), kidneys (apart from left renal small upper pole cyst), right adrenal, aorta, I.V.C and the pelvic organs.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic masses or collections.
- **The revised bone window revealed:**

1. Multilevel vertebral bodies tiny lucent foci.
2. Dorso-lumbar spondylo-degenerative changes.
3. No CT evidence of vertebral collapse or extra-osseous soft tissue component.

Opinion:

- **Right breast UOQ few enhancing nodules with pathologically enlarged right axillary as well as supraclavicular lymph nodes. For correlation with sonomammography findings.**
- **No definite pulmonary or hepatic metastatic deposits.**
- **Multilevel vertebral bodies tiny lytic foci for bone scan assessment.**

Reported By,
Toka Elbehairy,MSc
Dr. Mahmoud Ibrahiem, MD **18-Nov-2024 01:12 PM**

www.Baheya.org 16602

PATIENT NAME: Sabahah KHALEEFA NASR HASANEN	PATIENT ID: 10124253523
GENDER: Female	AGE: 48 y
DATE : 19-Nov-2024 05:37 PM	ORDER: 3130419,3130579,3130580

Dynamic Contrast Enhanced MRI Study of the Breast

Pulse Sequences:

- Pre-contrast: axial T1,T2,STIR and sagittal T2 WIs.
- DWI and ADC mapping.
- Post contrast: axial VIBE and Subtraction WIs

****complementary mammography and US were done.**

Indication:

- Pathological right axillary lymph nodes (pathologically proven metastatic of breast origin) to detect any occult breast lesions.

Report:

- Prominent fibroglandular parenchyma of both breasts showing mild nodular back ground parenchymal enhancement.

Right breast:

- Right breast outer quadrants (upper and lower) and deep retroareolar asymmetrical mild heterogenous parenchymal enhancement, more appreciated in delayed phases, corresponding to focal asymmetry on mammography. Complementary US revealed subtle parenchymal alteration with no definite masses.
- Right LOQ small circumscribed oval nodule with intrinsic fat signal measuring about 7x6mm?? IMLN.
- Multiple pathological partially amalgamated right level I,II and III axillary lymph nodes, the largest at level I measuring about 44x31mm. Multiple pathological supra clavicualr lymph nodes (seen by US).

Left breast:

- No enhancing lesions are seen, therefore, no time signal intensity curve was measured.
- Left UOQ tiny rounded rim enhancing nodule measuring about 6mm.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymphadenopathy.

www.Baheya.org
16602

Women's Health Outreach Program

Name: sabha khalifa nasr
Date: 14-10-2024

Bilateral Full field Digital Mammography

Clinical summary: Long standing history of right breast lump, which is now painful

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Heterogeneously dense breast parenchyma (ACR density c).
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Multiple pathologically enlarged axillary lymph nodes
- **Ultrasound** demonstrated No solid or cystic lesions.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- **Multiple enlarged axillary amgulated lymph nodes, all showing eccentric fatty hilum , the largest seen measuring about 4x3 c-n.**

Left breast

- Heterogeneously dense breast parenchyma (ACR density c).
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/subcutaneous tissue and nipple/areolar complex
- Non-specific axillary LNs.

Impression:

Right breast Normal breast study, with suspious axillary lymph nodes .
(BIRADS 4) for histopathology correlation (biopsy was taken)
Left breast (BIRADS 1): Normal study.

Best regards,
Dr. Norhan Osama , MD

PATIENT NAME: Sabahah KHALEEFA NASR HASANEN
GENDER: Female
DATE : 17-Nov-2024 02:30 PM

PATIENT ID: 10124253523
AGE: 48 y
ORDER: 3125992,3125993

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion.

FINDINGS:

1-CHEST

- Right breast UOQ multiple small enhancing nodules; the largest measures 7 mm in diameter.
- Multiple pathologically enlarged right axillary level I, II and fewer level III lymph nodes; the largest is at level I measuring 24 x 34 mm.
- Pathologically enlarged right supraclavicular lymph nodes; the largest measures 15 x 13 mm.
- Tiny right internal mammary lymph node is noted measuring 5 mm.
- Few subcentimetric left axillary level I and II and supraclavicular lymph nodes are noted.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathologically enlarged mediastinal or hilar lymph node.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- Mildly enlarged liver with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and is main branches as

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16602
مركز بهية للتدريب
٨ شارع طريبة أبو شارع الهرم بجوار دارى ، الجيزة
١ الحي الأول - الشيخ زايد أمام كلية الهندسة

pp

وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام

Echo report

Name: صاحبة خليفة
Age: ٤٠١٤
Date: ٧/٢ 2025 ٤٤

EF	٥٥	54-74%	PWT	٩	0.6-1.1cm
LVEDD	٤٩	3.5-5.7cm	AO	٣٢	2.0-3.7cm
LVESD	٣٣ ٣٥	2.2-4.0cm	LA	٣٩	1.9-4.0cm
FS		25-45%	RV		0.7-2.3cm (supine)
SWT	٩	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions:
LV wall thickness:
LV diastolic function:
SWMA.
LA size
Aortic root dimension
Mitral Valve
Aortic valve
Tricuspid valve
Pulmonary valve
Right side size and systolic function
Intracardiac masses and thrombi
Pericardium
IVC diameter and collapsibility.

- EF ٥٥٪

Conclusion:

D/Mostafa Shahien

Cairo University
National Cancer Institute
Department of Diagnostic and Interventional Radiology

جامعة القاهرة
المعهد القومي للأورام
قسم الأشعة التشخيصية والتداخلية

Exam Information:

Patient Name: ÔÇÉlâ ÍáíÝâ	***Date of birth:*** *12/07/1976*
Arabic Name: صفاء خليفه	***Patient Gender:*** *F*
Patient RIS ID: *365404* ***Patient HIS ID:*** *10110225002161*	***Exam Date:*** *03/02/2025*
Written by: Dr. Hoda Fathy	**Approved by:** Dr. Hoda Fathy

^{99m}Tc MDP Bone Scan Report (3/2/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP

Revealed:

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged

Mahmoud Akram
Omnia Talaat,MD

Patient Name	:الأستاذه / صابحه خليفه نصر حسانين	Request No.	:13008833
Patient No.	:27607012105707	Request Date	:15/10/2024
Path No.	:3933/24	Verification Date	:30/10/2024
Sex	:Female	Financial Category	:اورام دار السلام - نفقة الدولة
Age	:48 Y	Department	:Out Patient
Referreing Doc :		Registered by	:.marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.
Nature of Specimen: Tru-cut biopsy, collected at 14/10/2024, 11:00 am.

Gross Examination:
- Three grayish white tissue cores up to 1.2 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for P63, ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
* Histological type: Invasive duct carcinoma (Metaplastic carcinoma).
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+2) Grade III.
* Ductal carcinoma in situ: No.
* Perineural invasion: No.
* Angioinvasion: No.
* Additional pathologic findings: Focal squamoid differentiation.
* IPT (Positive control showed appropriate reactivity):
- P63: Focally positive in tumor cells.
- ER: No nuclear reaction in tumor cells (score 0/8).
- PR: Weak nuclear reaction in 1% of tumor cells (score 2/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 60% nuclear reaction of tumor cells.

Diagnosis:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade III (metaplastic carcinoma) with focal squamoid differentiation.
IPT:

Verified by :
Dr. Sagda El-Kady

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qedema - Cairo, Egypt

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

البنك المصري هم صحة مصر

MDT FORM 16 / 2 / 2025 G ٧٠٣

Name	[illegible]	**MRN**	
		ID	٢٠١٢/٤٠٤
Age	55 y	**Phone**	٠١١٤٨٦٤٧٥٢٢
Menopausal status	Post	**Marital status**	Married
NRS SCORE		**Family history**	-ve
BMI		**REASON OF MDT**	
CO morbidity	HTN		

Clinical Data

BX: IDC (metaplastic) grade III

ER:	PR:	HER 2:	Ki67:
018	218	0	60%

Sonomammography: ACR C
14/10/25 multiple enlarged LNs: BIRAD4
MRI: Path. LN+ RUOQ, Retroareolar BIRAD4 of parynchema -

Mets Workup: + enlarged supraclav.
CT CAP: vertebral bodies lytic bone lesions
for bone sc
Bone Scan: free

R L

P 63: Focally (+)



TNM

[signature]
٢٠/٤/٢٥

Recommendations:

MDT OUTCOME & PLAN

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: Mrs;Salha Khalifa
Date: 12 October 2024
Referred By: Prof. Dr.

RIGHT BREAST U/S EXAMINATION

- ***Multiple Right enlarged Axillary lymph nodes (Largest = 40x31 mm at their long.axis) , which are being rounded in shape with loss of their echogenic hilum showing peripheral vascularity(on CDS) . Two of them are matted together while the rest are discrete … likely related neoplastic process …….for histopathological correation***

- **The Right Breast :-**
  showing slightly echogenic glandular tissue of the Right breast with prominent dilated vascular structures of venous wave form on Doppler examination and no evidence o f dilated retroareolar ducts .

- *Normal nipple areola complex.*
- *Normal skin thickness of RIGHT breast .*

Thanks
DR. Mohamed Ibrahim

BI-RADS (Breast Imaging -Reporting And Data System)

1. Normal
2. Benign ……follow up/1 year
3. Probably benign……follow up/6months
4. Probably malignant ……for biopsy
5. Highly suggestive malignant……for surgery

GUSTAVE ROUSSY
INTERNATIONAL

Name: Sabha Khalifa
Date: 24/11/2025

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Findings:

Right breast.

- Heterogenously dense fibroglandular densities , ACR C
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast with mottled appearance with no definite solid or cystic lesions.
- **Increased skin thickness**
- Normal nipple/areola complex.
- Right pathological enlarged axillary lymph nodes (about 5) noted at level I and II showing suspicious criteria (some are rounded globular with lost fatty hilum , others show increased cortical thickness reaching 8 mm) , the largest is noted measuring 3.2 x 1.7 cm x 8 mm CT.
- Few pathologicaly enlarged right internal mammary lymph nodes.

Left breast.

- Heterogenously dense fibroglandular densities , ACR C
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast with mottled appearance with no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 6): A case of pathologically proven metastatic right breast carcinoma , Status follow up post NACT showing :

Multiple pathological right axillary and internal mammary lymphadenopathies with no definite right breast suspicious lesions.

Right breast post-therapeutic changes.

Left breast (BIRADS 1).

Israa Reda , MSc

Name : Sabha khalifa nasr hussnin	Date : 04-02-2026 02:25
Branch : Portable	Medical No. : GRI-27607012105707
RefPhysician Prof : cap+c	Ins.Company :

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

A known case of right breast cancer for metastatic workup

I- CHEST:

- The right breast showed retro-areolar soft tissue mass lesion measures about 5.4x4 cm with overlying skin thickness measures about 8 mm. associated with multiple enlarged ipsilateral axillary lymph nodes, the largest measures about 3.4x2.5 cm with central necrosis **for further sono-mammography**
- Bilateral basal atelectatic bands are noted.
- No pulmonary nodules, masses or cavitary lesions.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Average cardiac size
- Bilateral fibroelastoma dorsi being evident on right side.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with single stone measures about 1.4 cm.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.

Name : Sabha khalifa nasr hussnin	**Date : 04-02-2026 02:25**
Branch : Portable	**Medical No. : GRI-27607012105707**
Ref.Physician Prof : captc	**Ins.Company :**

- No abdominal or pelvic lymphadenopathy.
- Pelvic calcific phleboliths are noted
- No ascites.
- **The examined skeleton** no worrisome lytic or sclerotic lesions.

Opinion:

- **Bilateral basal atelectatic bands.**
- **Single gall bladder stone.**

Dr.Norhan Ramzy,MD

Dr.NOURHAN RAMZY

Dr.NOURHAN RAMZY

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصرية هي صحة مصر

VYEE

MDT FORM 5/11/2025 G

Name	ماريت خليفة نادر	MRN	E.18/C1EE
		ID	10124253523
Age	55 yrs old	Phone	01140433568
Menopausal status	Post menopausal	Marital status	Married
NRS SCORE		Family history	-ve
BMI		REASON OF MDT	cT2.N1.M0.IIB
CO morbidity	HTN & DM		

Clinical Data

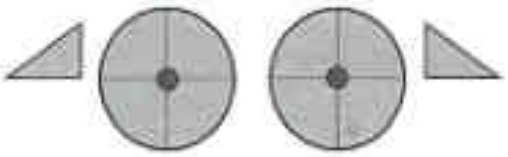
BX: IDC GIII

ER:	PR:	HER 2:	Ki67:
0/8	2/8	-ve	60%

Sonomammography:
Rt Br normal study 10/2024
+ suspicious AUNs

Mets Workup: MRI Rt Br → Rt Br Retro Parenchy (suspicious) lesion
- CT CAP → multiple lytic lesions → Path. enlarged supraclavicular LN 15x13 mm

R L



TNM
03c

Recommendations: Bone scan → free
Pt received NACT DD AC x4 & Taxol/Carbo x12 weeks
for surgical Review regarding Rt MRM & plan of management
as per last MDT (Surgical Review)
26/10/2025
For new Sonomammo to assess Response
+ supra clavi LN
Still skin edema + palp axillary LN, then consider Rt MRM

MDT OUTCOME & PLAN

Last cycle: 26/10/2025. Omnitarg 26/10/2025

For new Sonomammography.

NEW CAPS SIGNATURE if Not metastatic for Rt MRM

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
		Mona Tolly			

GUSTAVE ROUSSY
INTERNATIONAL

Name: Sabha Khalifa
Date: 24/11/2025

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- Few pathologicaly enlarged right internal mammary lymph nodes.

Left breast.

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Left breast (BIRADS 1).

Israa Reda , MSc

وستاف وسي ناشونال

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Branch : Portable
Ref.Physician Prof : cap+c

Date : 04-02-2026 02:25
Medical No. : GRI-27607012105707
Ins.Company :

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Name : Sabha khalifa nasr hussnin	**Date : 04-02-2026 02:25**
Branch : Portable	**Medical No. : GRI-27607012105707**
Ref.Physician Prof : cap+c	**Ins.Company :**

- No abdominal or pelvic lymphadenopathy.
- Pelvic calcific phleboliths are noted
- No ascites.
- **The examined skeleton** no worrisome lytic or sclerotic lesions.

Opinion:

- **Bilateral basal atelectatic bands.**
- **Single gall bladder stone.**

Dr.Norhan Ramzy,MD

Dr.NOURHAN RAMZY

Dr.NOURHAN RAMZY

Patient Name	:الأستاذه / صفيحه خليفه نصر حسانين	Request No.	:13008833
Patient No.	:27607012105707	Request Date	:15/10/2024
Path No.	:3933/24	Verification Date	:30/10/2024
Sex	:Female	Financial Category	:اورام دار السلام - نفقة الدولة
Age	:48 Y	Department	:Out Patient
Referreing Doc :		Registered by	:.marwa mohamed youss

Pathology Report

ERM036728263

- P63: Focally positive in tumor cells.
- ER: Negative (score 0/8).
- PR: Negative (score 2/8).
- Her2: Negative (score 0).
- Ki67: Positive in 60% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Rana Fathy.
Dr. Sagda Elkady.

Verified by :
Dr. Sagda El-Kady

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Preventive Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : صابحه شفيقه نصر حسانين
Sex: Female
3- Medical number: 83243886
Old Med :
4012-2024

2-ID number:
Patient Age : 48 Y

Family history	no	Phone	01140433168	BMI		REASON OF MDT	
marital status		NRS Score		GROUP		Date of MDT	2025-02-27
Co morbidity	htn	Menopausal status		Initial STAGE			

Clinical data

Breast Imaging :
rt breast normal study + suspicious LN

Mets Workup :
CTCAP FREE
mri breast: pathological LN rt breast outer &
Retro areolar quadrants suspicious parenchyma, LEVEL I
LEVEL II, III AXILLA , SUPRACLAV
BONE SCAN FREE

BX	ER	PR	Her 2 Neu	SISH	KI67
IDC gIII	0/8	2/8			60

T	N	M
CTX ALTERED PARENCHYMA,	3C	0

Sonomammography

PATIENT HISTORY
ECHO, LABS

MDT OUTCOME & PLAN

NACT DDACX4 WITH GCSF SUPPORT THEN TAXOL CARBOX12 FOLLOWED BY RT MRM

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Sabha Khalifa Nasr Hassanain | Date : 05-10-2026 02:21
Branch : Dokki | Medical No. : 477180
Ref.Physician Prof : Goustav Roussey Egypt

Tc-99m MDP WHOLE-BODY BONE SCINTIGRAPHY REPORT

Clinical Indication: Follow-up for skeletal metastatic disease in a 50-year-old female with right breast carcinoma diagnosed two years ago, post-surgical resection and chemotherapy, presenting with right arm and head pain.

Procedure: Whole-body anterior and posterior planar bone scintigrams were acquired following intravenous Tc-99m MDP administration, with static lateral views of the skull.

FINDINGS

- A focal area of intensely increased radiotracer uptake is clearly demonstrated in the right frontoparietal calvarium, well delineated on the anterior, posterior, and lateral views, strongly suspicious for an active osteoblastic metastatic deposit.
- Physiological radiotracer distribution is otherwise maintained throughout the cervical, thoracic, and lumbar spine, pelvic girdle, rib cage, and remaining appendicular long bones, without additional hypermetabolic osseous lesions in the right upper limb or remainder of the skeleton.
- Clear soft tissue background excretion with prompt bilateral renal activity and physiological bladder accumulation.

IMPRESSION

- Solitary intense focal hypermetabolic lesion in the right frontoparietal calvarium, highly suspicious for an active osteoblastic metastatic deposit, for clinical and radiological correlation.
- No other skeletal lesions identified.

Much obliged
Dr. Alaa Nagy, MD
Nuclear medicine

NILE DIAGNOSTIC CENTER